

Redacted



Gender: Male

DOB:

Race:

Report Date:

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

- A. Right distal ureter; biopsy:
 - Segment of ureter, no evidence of tumor or dysplasia.
- B. Left pelvic lymph nodes; dissection:
 - Two of eight lymph nodes positive for metastatic urothelial carcinoma (2/8) with extranodal extension.
- C. Right pelvic lymph nodes:
 - Two of thirteen lymph nodes positive for metastatic urothelial carcinoma (2/13) with extranodal extension.
- D. Left distal ureter; biopsy:
 - Segment of ureter, no evidence of tumor or dysplasia.
- E. Left pelvic lymph nodes; dissection:
 - One of eight lymph nodes positive for metastatic urothelial carcinoma (1/8).
- F. Bladder, prostate, seminal vesicles, vas deferens; radical cystoprostatectomy:
 - High-grade urothelial carcinoma, see pathologic parameters.
 - Ulcerated mucosa with marked granulomatous reaction consistent with therapy effect.
 - Adenocarcinoma of prostate, see below.

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
IPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Notes		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed Initial	Reviewed	Reviewed

Urothelial Carcinoma

1. Tumor type: Urothelial carcinoma
2. Grade of tumor: High-grade
3. Depth of invasion: Extravesicular fat
4. Tumor distribution:
 - Solitary, 3.8 cm in size
 - left lateral wall
 - posterior wall
5. Ureteral Margins: Negative
6. Distal urethral margin: Negative
7. Lymph nodes: Five of 29 lymph nodes positive for metastatic urothelial carcinoma
8. pTNM: pT3a, N2, MX

ICD-O-3

Carcinoma, urothelial, NOS

8120/3

Site:

Path: bladder, NOS

C67.9

CQCF: bladder, wall, lateral

C67.2

4-9-12

RO

Radical Prostatectomy Pathologic Parameters

1. Gleason's Grade: 6 (3+3)
2. Perineural invasion: Absent
3. Tumor location:
 - Peripheral zone - right
4. Tumor Volume Estimate: 1%

5. High-grade P.I.N.: Multifocal
6. Seminal Vesicles: Negative for tumor
7. Fibromuscular capsule: Tumor confined to the prostate
8. Peripheral margin: Negative for tumor
9. Distal (apical) Margin: Negative for tumor
10. Proximal (basilar) margin: Negative for tumor
11. Regional Lymph Nodes (right and left): Negative for prostatic tumor
12. pTNM: pT2a, N0, MX

xxx

[], M.D.

Interpretation performed by the Attending Pathologist and reviewed with the Resident or Fellow.

Electronically Signed Out by [] M.D.

Clinical History:

The patient is a year-old male with bladder cancer who undergoes radical cystectomy and bilateral pelvic lymph node dissection.

Specimens Received:

- A: Right Distal Ureter
- B: Left Pelvic Lymph Nodes
- C: Right Pelvic Lymph Nodes
- D: Left Distal Ureter
- E: Left Pelvic Lymph Nodes
- F: Bladder, Prostate, Seminal Vesicles, Vas Deferens

Gross Description:

The specimen is received in six containers, each labeled with the patient name and medical record number.

A. Part A is further designated as "1. right distal ureter." Received fresh for frozen section is a 0.7 cm segment of pink-tan tissue with a 0.5 cm lumen, consistent with a ureter. The specimen is serially sectioned and partially submitted for frozen section, which is read as "segment of ureter, no evidence of tumor and no dysplasia" per Dr. []. The remainder of the specimen is submitted in cassette A2.

B. Part B is further designated as "2. left pelvic lymph nodes." Received fresh for frozen section is a 4.0 x 3.0 x 1.0 cm aggregate of pink-tan, fibrofatty tissue. Two lymph nodes are submitted for frozen section and read as "one lymph node with microscopic focus (<2 mm) of metastatic urothelial carcinoma, another lymph node is benign, reactive" per Dr. [] The frozen section remnant is submitted in cassette B1FS. The remainder of the specimen is submitted entirely as follows:

- B2: one large lymph node, bisected
- B3: one large lymph node, bisected
- B4: three small lymph nodes
- B5-B6: remaining fibroadipose tissue

C. Part C is further designated as "3. right pelvic lymph nodes." Received fresh and placed in formalin is a 7.0 x 5.0 x 2.0 cm aggregate of fibrofatty

tissue. Multiple lymph nodes are identified upon dissection, the largest measuring 4.0 x 3.0 x 0.2 cm. The lymph nodes are submitted as follows:

C1-C11: one large lymph node in each cassette, some are bisected and/or serially sectioned

C12: four lymph nodes

D. Part D is further designated as "4. left distal ureter." Submitted fresh for frozen section is a 0.9 cm length of pink-tan, rubbery tissue with a 0.6 cm external diameter and a 0.3 cm lumen. The specimen is bisected and submitted for frozen section. The frozen section is read as "ureteric segment with no evidence of tumor and no dysplasia" by Dr. [] The remainder of the frozen section is submitted in cassette D1FS. The remaining specimen is submitted in cassette D2.

E. Part E is further designated as "5. left pelvic lymph nodes." Received in formalin is a 5.0 x 4.0 x 3.0 cm aggregate of soft, fibrofatty tissue, which is dissected to reveal multiple lymph nodes. The specimen is submitted as follows:

E1-E4: one large lymph node in each cassette, bisected

E5: four possible lymph nodes

E6: four possible lymph nodes

F. Part F is further designated as "6. bladder, prostate, seminal vesicles, vas deferens." Received fresh and placed in formalin is a 9.5 x 8.0 x 5.5 cm cystoprostatectomy specimen with attached mesenteric fat. The attached prostate measures 4.0 x 3.8 x 2.9 cm. The left seminal vesicle measures 4.0 x 1.5 cm and the left vas deferens measures 9.5 cm. The right seminal vesicle measures 3.5 x 1.8 cm and the right vas deferens measures 9.0 cm. The entire specimen is inked blue. The prostate is opened along the urethra and reveals tan-brown, wrinkled mucosa. The bladder itself measures 6.0 x 5.0 x 3.8 cm. The right ureteral stump measures 2.4 cm and has a patent lumen. The left ureteral stump measures 2.0 cm and also has a patent lumen. There is a 3.8 x 2.6 cm ulcerated, red-tan lesion on the left posterolateral wall, reaching from the left trigone to the bladder dome. Grossly, the mass extends into the prostatic urethra and the left half of the prostate. The right ureteral orifice at the trigone can be probed, the left cannot. Sectioning of the mass shows gross tumor invasion of the mucosal wall, 1.0 cm from the deep margin. The mucosa of the anterior wall is glistening and pink-gray in color. Sectioning of the prostate reveals trabeculated, pink-tan periurethral nodularity. No lesions are grossly identified. Representative sections are submitted as follows:

F1: distal resection margin

F2-F4: representative section of lesion from left posterolateral wall

F5: representative lesion in bladder dome

F6: right posterior bladder wall

F7: right anterior bladder wall

F9: anterior bladder wall

F10: right inferior bladder wall

F11-F15: representative sections of the left prostate

F16-F20: representative sections of the right prostate

F21: seminal vesicles and vas deferens

F22: distal ureter

F23: distal vas deferens

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[] M.D.

Intraoperative Consult Diagnosis:

A1FS. Right distal ureter: Segment of ureter, no evidence of tumor and no dysplasia.

B1FS. Left pelvic lymph nodes: One lymph node with microscopic focus (<2 mm) of metastatic urothelial carcinoma, another lymph node is benign, reactive.

D1FS. Left distal ureter: Ureteric segment with no evidence of tumor and no dysplasia.